Hunter Valley Base Hospital — Discharge Summary

300 Vineyard Drive, Hunter Valley Central NSW 2330 | Tel: (02) 4998 2000 | Fax: (02) 4998 2001
Hunter New England Local Health District | NSW Health

Patient Name:	**Gregory Raj Conti**	Medical Record No.:	**895263**
Date of Birth:	26/07/1937 (Age 88)	Medicare No.:	5510 30224 7
Address:	160 River Tce, Leichhardt NSW 2040	Next of Kin:	Priya Conti (Wife) 0443 501 921
GP:	Dr Richard Hussein, Bayside Medical Centre	Allergies:	**Iodine contrast (mild rash)**

ADMISSION DETAILS

Date Admitted:	13/09/2025 via Emergency Dept	Date Discharged:	15/09/2025
Ward:	5B (Cardiology)	Length of Stay:	2 days
Admitting Clinician:	Dr H. Nandakumar (Advanced Trainee, Cardiology)	Consultant:	A/Prof M. Whitfield (Staff Specialist)
Discharge Destination:	Home with community nursing	Discharge Mode:	Self / family transport

PRESENTING COMPLAINT

Mr Conti attends for cardiology review with 6-month Past medical history of gradually gradually worsening dyspnoea on exertion. Also, chest tightness. Episode of near-syncope on stairs a 2-week history ago. Now limited to 50m walking.

PRINCIPAL DIAGNOSIS

Severe aortic stenosis with progressive exertional symptoms (ICD-10-AM: I35.0)

ADDITIONAL DIAGNOSES

1. Hypertension
2. Atrial fibrillation (paroxysmal)
3. Mild chronic kidney disease

HOSPITAL COURSE

Admitted for elective transcatheter aortic valve implantation (TAVI) workup. Heart team review confirmed appropriate for TAVI. Pre-procedure CT and angiogram completed. Procedure performed without complication; transfemoral approach. Discharged Day 3 post-procedure on dual antiplatelet therapy.

MEDICATIONS AT DISCHARGE

Medication	Dose / Route / Frequency	Notes
Aspirin	100mg PO mane	NEW - lifelong
Clopidogrel	75mg PO mane	NEW - 6 months
Apixaban	5mg PO BD	Unchanged - paroxysmal AF
Bisoprolol	5mg PO mane	Unchanged
Furosemide	40mg PO mane	Unchanged

KEY INVESTIGATIONS

Bloods on admission (13/09/2025): BNP 420 pg/mL (H), Creatinine 108 umol/L, Hb 118 g/L (L).

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Bloods at discharge (15/09/2025): Hb 104 g/L (L).

Imaging: TTE: severe aortic stenosis (peak velocity 4.6 m/s, mean gradient 58mmHg, AVA 0.7cm^2). LVEF 50% with mild concentric hypertrophy. Post-TAVI TTE: well-seated valve, mean gradient 12mmHg, mild paravalvular leak.

FOLLOW-UP PLAN

1. Cardiology outpatient review at 6 weeks.
2. GP review at 1 week.
3. Repeat TTE at 30 days post-TAVI.
4. Continue clopidogrel for 6 months.

ALERTS AND PRECAUTIONS

ALLERGY: Iodine contrast (mild rash)
TAVI valve in situ. Endocarditis prophylaxis for dental procedures.

PREPARED BY

Dr Hamish Nandakumar, Advanced Trainee, Cardiology

Reviewed and approved by: A/Prof Matthew Whitfield, Staff Specialist, Cardiology

Date: 15/09/2025

Dr H. Nandakumar (Advanced Trainee) A/Prof M. Whitfield (Staff Specialist)

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